



Drug Management Policy

Tremfya®

Policy Number: 9.150

Version Number: R4

Version Effective Date: 3/1/2025

Impacted Products

- ☐ **All Products**
- ☒ NH Medicaid
- ☐ NH Medicare Advantage
- ☐ MA MassHealth ACO
- ☐ MA MassHealth MCO
- ☐ MA Qualified Health Plans/Employer Choice Direct
- ☐ MA Senior Care Options

Applicable Benefit

- ☒ Pharmacy
- ☐ Medical

Note: Disclaimer and audit information is located at the end of this document.

Prior Authorization Policy

Products Affected:

- **Tremfya (guselkumab)**

The Plan may authorize coverage of the above products for members meeting the following criteria:

Covered Use	All FDA approved indications unless otherwise excluded
Exclusion Criteria	Use in combination with other biologics
Required Medical Information	Moderate to severe plaque psoriasis (PsO) 1. The member is 18 years of age or older; And 2. The requested medication is prescribed by or in consultation with a dermatologist; AND 3. One of the following: a. Involvement of at least 3% of total body surface area prior to the use of the requested medication; OR b. Hands, feet, scalp, face, or genital area affected; AND 4. Documentation of one of the following: a. An inadequate response or adverse reaction to at least a 3 month trial of ONE biologic DMARD that is FDA-approved for plaque psoriasis; OR b. An inadequate response or adverse reaction to at least a 3 month trial of any ONE of the following combinations (please note: these combinations DO NOT have to be used concurrently): i. one topical agent plus one systemic agent; OR ii. one topical agent plus one phototherapy; OR iii. one systemic agent plus one phototherapy; OR iv. two systemic agents; OR v. a contraindication to ALL conventional therapies (topical agents, phototherapy, and systemic agents); AND 5. Documentation of ONE of the following: a. An inadequate response (to at least 3 months of treatment) or adverse reaction to ONE of the following PDL preferred agents: Enbrel, , a preferred adalimumab biosimilar product (adalimumab-aacf, adalimumab-adaz, adalimumab-adbm, adalimumab-fjkg, adalimumab-ryvk) OR Humira,), infliximab (generic for Remicade), Otezla, or Taltz; OR b. A contraindication to ALL of the following: Enbrel, adalimumab, infliximab, Otezla, AND Taltz AND

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6. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A).

Psoriatic Arthritis (PsA)

1. The member is 18 years of age or older; And
2. The requested medication is prescribed by or in consultation with a dermatologist or rheumatologist; **AND**
3. ONE of the following:
 - a. An inadequate response or adverse reaction to at least a 3 month trial of ONE non-biologic disease modifying antirheumatic drug (DMARD) or contraindication to ALL non-biologic DMARDs; **OR**
 - b. An inadequate response or adverse reaction to at least a 3 month trial of ONE biologic DMARD that is FDA-approved for psoriatic arthritis; **AND**
4. Documentation of ONE of the following:
 - a. An inadequate response (to at least 3 months of treatment) or adverse reaction to ONE of the following PDL preferred agents: Enbrel, a preferred adalimumab biosimilar product (adalimumab-aacf, adalimumab-adaz, adalimumab-adbm, adalimumab-fjkg, adalimumab-ryvk) OR Humira, infliximab (generic for Remicade), Otezla, Rinvoq, Taltz, or Xeljanz;; **OR**
 - b. A contraindication to ALL of the following: Enbrel, adalimumab, infliximab, Otezla, Rinvoq, Taltz, AND Xeljanz; **AND**
5. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A).

Coverage Duration	Initial: 6 months Reauthorization: 12 months
Other criteria	Reauthorization: 1. Initial criteria has been met; AND 2. One of the following: a. The member has had a positive response to therapy; OR

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- b. Documentation of clinical justification to continue therapy with the requested medication

Appendix A – FDA Approved Dosing

Diagnosis	FDA Approved Dosing
Plaque Psoriasis	100 mg injection at Week 0, Week 4 and every 8 weeks thereafter.
Psoriatic Arthritis	100 mg injection at Week 0, Week 4 and every 8 weeks thereafter.

Appendix B Non-Biologic DMARD Treatment Options

Diagnosis	Non-Biologic DMARD Treatment Options
Plaque psoriasis	Methotrexate Azathioprine Cyclosporine
Psoriatic Arthritis	Methotrexate Leflunomide Sulfasalazine Azathioprine

Note: other trials may be considered on a case-by-case basis

Applicable Coding

Code	Medication
J1628	Injection, guselkumab, 1 mg

Clinical Background Information and References

1. Elmetts, C. Korman, N. Farley Prater, E. et. al. Joint AAD–NPF Guidelines of care for the management and treatment of psoriasis with topical therapy and alternative medicine

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modalities for psoriasis severity measures. Journal of the American Association of Dermatology. 84(2):p432-470. Feb. 2021. Accessed July 2024.

2. Elmetts, C. Lim, H, Wu, J. et. al. Joint American Academy of Dermatology–National Psoriasis Foundation guidelines of care for the management and treatment of psoriasis with phototherapy. Journal of the American Association of Dermatology. 81(3):p775-804. Sept. 2019. Accessed July 2024.
3. Menter A, Strober BE, Kaplan DH, et al. Joint AAD-NPF guidelines of care for the management and treatment of psoriasis with biologics. J Am Acad Dermatol. 2019;80(4):1029-1072. Accessed July 2023=4.
4. Singh JA, Guyatt G, Ogdie A, et al. 2018 American College of Rheumatology/National Psoriasis Foundation Guideline for the Treatment of Psoriatic Arthritis. Arthritis & Rheumatology. 2019; Jan;71(1):5-32.
5. Tremfya (guselkumab) [prescribing information]. Horsham, PA: Janssen Biotech Inc. July 20234

Policy History

Original Approval Date	Original Effective Date	Policy Owner	Approved by
8/11/2022	9/01/2022	Pharmacy Services	Pharmacy & Therapeutics (P&T) Committee, NH DHHS

Policy Revisions History			
Review Date	Summary of Revisions	Revision Effective Date	Approved by
8/11/2022	P&T: New policy created	9/01/2022	P&T Committee, NH DHHS
8/19/2022	Updated to reflect new PDL preferred agents in trial and failure requirements	9/1/2022	P&T Committee, NH DHHS
08/10/2023	Annual P&T review: Require documentation of the use of required	12/1/2023	P&T Committee, NH DHHS

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Policy Revisions History

	trial/failure agents prior to approval. Addition of dosing criteria and appendix. Updated reauthorization criteria to a standard requirement for all inflammatory disease agents.		
5/9/2024	PDL Alignment: Minor formatting changes. In the revised policy, preferred adalimumab biosimilar products are included as alternatives for diagnoses of Plaque Psoriasis, and Psoriatic Arthritis.	9/1/2024	P&T Committee, NH DHHS
8/8/2024	P&T annual review: no recommended changes.		P&T Committee
9/1/2024	Policy updated to reflect Rinvoq as preferred product on PDL.		P&T Committee
11/26/2024	Policy updated to reflect a new adalimumab as preferred drug on PDL	3/1/2025	P&T Committee, NH DHHS

Next Review Date

8/2025

Other Applicable Policies

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Reference to Applicable Laws and Regulations, If Any

Disclaimer Information

Medical Policies are the Plan's guidelines for determining the medical necessity of certain services or supplies for purposes of determining coverage. These Policies may also describe when a service or supply is considered experimental or investigational, or cosmetic. In making coverage decisions, the Plan uses these guidelines and other Plan Policies, as well as the Member's benefit document, and when appropriate, coordinates with the Member's health care Providers to consider the individual Member's health care needs.

Plan Policies are developed in accordance with applicable state and federal laws and regulations, and accrediting organization standards (including NCQA). Medical Policies are also developed, as appropriate, with consideration of the medical necessity definitions in various Plan products, review of current literature, consultation with practicing Providers in the Plan's service area who are medical experts in the particular field, and adherence to FDA and other government agency policies. Applicable state or federal mandates, as well as the Member's benefit document, take precedence over these guidelines. Policies are reviewed and updated on an annual basis, or more frequently as needed. Treating providers are solely responsible for the medical advice and treatment of Members.

The use of this Policy is neither a guarantee of payment nor a final prediction of how a specific claim(s) will be adjudicated. Reimbursement is based on many factors, including member eligibility and benefits on the date of service; medical necessity; utilization management guidelines (when applicable); coordination of benefits; adherence with applicable Plan policies and procedures; clinical coding criteria; claim editing logic; and the applicable Plan – Provider agreement.

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